

D405

Financial Resource Management and Healthcare Reimbursement

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A 90-physician group sees its claim denials rise to 12%. Most denials say “patient not eligible on date of service” or “no prior authorization.” The group just moved to a new office building. Where should the fix focus?

- A. Hire more coders.
- B. Front-end registration: verify eligibility and get prior authorization before visits.
- C. Wait until the office move settles.
- D. Resubmit all denied claims faster.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A primary care group is paid \$40 per member per month for 5,000 members. Its founder was named a top doctor last year. This winter, member visits rise 30% because of flu. What happens to the group’s revenue from this contract?

- A. It doubles because of the founder’s reputation.
- B. It stays the same, while costs rise; the group bears the risk.
- C. It rises 30% with visits.
- D. It falls because the payer bears the risk.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3. [TRANSFER — reversed stem / different variable]

Under which payment model does the payer bear most of the risk of higher service use?

- A. Capitation
- B. Bundled payment
- C. Global budget
- D. Fee-for-service

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A hospital receives a DRG payment of \$12,000 for a pneumonia admission. The patient stays 5 days at a cost of \$2,800 per day. The hospital was recently rated four stars. What is the hospital's gain or loss?

- A. A \$2,000 gain
- B. \$14,000 in revenue
- C. A \$9,200 gain
- D. A \$2,000 loss

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A health plan pays a clinic \$45 per member per month for 2,000 members. The clinic had 9,500 visits last year and has two locations. What is its annual capitation revenue?

- A. \$7,500
- B. \$1,080,000
- C. \$90,000
- D. \$427,500

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A clinic wants \$1,200,000 a year from a capitation contract covering 2,500 members. What PMPM rate does it need?

- A. \$100,000
- B. \$480
- C. \$3.33
- D. \$40

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A hospital has \$4,500,000 in accounts receivable and \$36,500,000 in annual net patient revenue. It has 250 beds. What are its days in A/R?

- A. 45 days
- B. 8.1 days
- C. 12.3%
- D. 18,000 days

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8. [TRANSFER — reversed stem / different variable]

A clinic's daily net revenue is \$20,000. Its goal is 40 days in A/R. What A/R balance meets the goal?

- A.** \$20,000
- B.** \$7,300,000
- C.** \$800,000
- D.** \$500

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A billing office submitted 5,000 claims last month; 400 were denied, and 4,600 were paid. The office uses a cloud-based system installed last year. What was the denial rate?

- A.** 40%
- B.** 92%
- C.** 8%
- D.** 8.7%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A hospital's chargemaster shows \$2,000,000 in gross charges for the month. Contractual adjustments with insurers total \$800,000. The hospital just upgraded its billing software. What is net patient revenue?

- A.** \$1,200,000
- B.** \$2,000,000
- C.** \$2,800,000
- D.** \$800,000

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11. [TRANSFER — reversed stem / different variable]

A clinic reports \$600,000 in net patient revenue after \$400,000 in contractual adjustments. What were gross charges?

- A.** \$1,000,000
- B.** \$1,400,000
- C.** \$200,000
- D.** \$600,000

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12.

A coder at a busy practice routinely bills every office visit at the highest level, even when notes show a brief follow-up. The practice manager says payers rarely question it, and the practice has 15 physicians. What is the problem?

- A.** A HIPAA violation.
- B.** None, since payers rarely object.
- C.** Upcoding — a fraud risk under the False Claims Act.
- D.** Unbundling

Target word	The one fact that decides	Answer	☐ sure ☐ unsure ☐ guess
_____	_____	____	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

A 90-physician group sees its claim denials rise to 12%. Most denials say “patient not eligible on date of service” or “no prior authorization.” ~~The group just moved to a new office building.~~ Where should the fix focus?

Target word: where to fix **Decides:** Eligibility and authorization errors.

A	Adjacent category	The denial reasons are not coding errors.
B	Correct	These denials start at the front end.
C	Camouflage	The move is irrelevant.
D	Inverted operation	Treats symptoms after the fact.

2. Answer B Concept 2

A primary care group is paid \$40 per member per month for 5,000 members. ~~Its founder was named a top doctor last year.~~ This winter, member visits rise 30% because of flu. What happens to the group’s revenue from this contract?

Target word: risk under capitation **Decides:** Fixed PMPM.

A	Camouflage	Reputation doesn’t change contract revenue.
B	Correct	Capitation revenue does not change with volume.
C	Inverted operation	That is fee-for-service logic.
D	Confusion set	Under capitation, the provider bears risk.

3. Answer D Concept 2 · transfer

Under which payment model does the payer bear most of the risk of higher service use?

Target word: reverse: from risk to model **Decides:** Pays per service.

A	Inverted operation	Provider bears risk.
B	Confusion set	Risk is shared for an episode.
C	Adjacent category	A fixed budget also shifts risk to the provider.
D	Correct	Each extra service is paid for.

4. Answer D Concept 3

A hospital receives a DRG payment of \$12,000 for a pneumonia admission. The patient stays 5 days at a cost of \$2,800 per day. ~~The hospital was recently rated four stars.~~ What is the hospital’s gain or loss?

Target word: profit/loss under DRG **Decides:** Cost \$14,000 vs. fixed \$12,000.

A	Inverted operation	Right size, wrong direction.
B	Wrong stem number	That is the cost of the stay; DRG payment is fixed at \$12,000.
C	Dropped element	$\$12,000 - \$2,800$ counts only one day of cost.
D	Correct	$5 \times \$2,800 = \$14,000$; $\$12,000 - \$14,000 = -\$2,000$.

5. Answer B Concept 4

A health plan pays a clinic \$45 per member per month for 2,000 members. ~~The clinic had 9,500 visits last year and has two locations.~~ What is its annual capitation revenue?

Target word: compute capitation revenue **Decides:** $\$45 \times 2,000 \times 12$.

A	Inverted operation	$\$90,000 \div 12$ divides instead of multiplies.
B	Correct	$\$45 \times 2,000 = \$90,000$ per month $\times 12 = \$1,080,000$.
C	Dropped element	Leaves out the 12 months.
D	Camouflage	$\$45 \times 9,500$ visits uses the visits, which don't decide capitation.

6. Answer D Concept 4 • transfer

A clinic wants \$1,200,000 a year from a capitation contract covering 2,500 members. What PMPM rate does it need?

Target word: solve for rate **Decides:** $\$1,200,000 \div 2,500 \div 12$.

A	Wrong stem number	Monthly revenue, not per member.
B	Dropped element	Annual rate per member; leaves out the months.
C	Inverted operation	Divides by 12 twice.
D	Correct	$\$1,200,000 \div 2,500 = \480 per year $\div 12 = \$40$.

7. Answer A Concept 5

A hospital has \$4,500,000 in accounts receivable and \$36,500,000 in annual net patient revenue. ~~It has 250 beds.~~ What are its days in A/R?

Target word: compute days in A/R **Decides:** Daily revenue \$100,000.

A	Correct	$\$36,500,000 \div 365 = \$100,000$; $\$4,500,000 \div \$100,000 = 45$.
B	Inverted operation	$\$36,500,000 \div \$4,500,000$ is A/R turnover, not days.
C	Dropped element	$\$4,500,000 \div \$36,500,000$ drops the conversion to daily revenue.
D	Camouflage	$\$4,500,000 \div 250$ beds uses the bed count.

8. Answer C Concept 5 • transfer

A clinic's daily net revenue is \$20,000. Its goal is 40 days in A/R. What A/R balance meets the goal?

Target word: solve for A/R **Decides:** $A/R = \text{days} \times \text{daily revenue}$.

A	Dropped element	One day of revenue.
B	Wrong stem number	Annual revenue ($\$20,000 \times 365$).
C	Correct	$40 \times \$20,000 = \$800,000$.
D	Inverted operation	Divides instead of multiplies.

9. Answer C Concept 6

A billing office submitted 5,000 claims last month; 400 were denied, and 4,600 were paid. ~~The office uses a cloud-based system installed last year.~~ What was the denial rate?

Target word: compute denial rate **Decides:** $400 \div 5,000$.

A	Wrong stem number	Misreads 400 as a percentage.
B	Inverted operation	The share not denied.
C	Correct	$400 \div 5,000 = 8\%$.
D	Confusion set	$400 \div 4,600$ uses paid claims as the base.

10. Answer A Concept 7

A hospital's chargemaster shows \$2,000,000 in gross charges for the month. Contractual adjustments with insurers total \$800,000. ~~The hospital just upgraded its billing software.~~ What is net patient revenue?

Target word: compute net revenue **Decides:** Gross – contractual adjustments.

A	Correct	$\$2,000,000 - \$800,000$.
B	Dropped element	Treats gross charges as revenue.
C	Inverted operation	Adds the adjustments.
D	Wrong stem number	The adjustments themselves.

11. Answer A Concept 7 • transfer

A clinic reports \$600,000 in net patient revenue after \$400,000 in contractual adjustments. What were gross charges?

Target word: solve for gross charges **Decides:** Net + adjustments.

A	Correct	$\$600,000 + \$400,000$.
B	Wrong stem number	Adds \$400,000 twice.
C	Inverted operation	Subtracts instead of adds.
D	Dropped element	Treats net as gross.

12. Answer C Concept 8

A coder at a busy practice routinely bills every office visit at the highest level, even when notes show a brief follow-up. The practice manager says payers rarely question it, ~~and the practice has 15 physicians.~~ What is the problem?

Target word: compliance issue **Decides:** Code level exceeds documentation.

A	Adjacent category	Privacy law, not coding.
B	Absolute language	Lack of challenge does not make it legal.
C	Correct	Codes must match documentation.
D	Confusion set	Unbundling splits a bundled service into parts.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	7	
Wrong stem number	A figure copied from the prompt that is not the answer.	6	
Adjacent category	The right idea in the wrong bucket.	3	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	11	
Absolute language	Always, never, guarantee — where judgment applies.	1	
Confusion set	Every option is real; the test is exact role-matching.	4	
Camouflage	Answers a stem fact that was never the question.	4	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Revenue cycle front end	Most denials are prevented at the front end: verify eligibility and benefits, obtain prior authorization, and capture accurate demographics before the service.	Thinking denials are mainly a back-end billing problem fixed by resubmitting.	Denials traced to coverage, authorization, or registration errors.
2	Who bears the financial risk	Fee-for-service pays per service, so the payer bears utilization risk; capitation pays a fixed amount per member per month, so the provider bears it; bundled payments share risk for an episode.	Thinking capitated providers earn more by doing more services.	A payment model plus a change in service volume.
3	DRG prospective payment	Under a DRG system, the hospital gets a fixed payment per admission based on diagnosis, so profit or loss depends on keeping costs below that payment.	Assuming longer stays raise a DRG payment.	A fixed DRG amount with daily costs and a length of stay.
4	Capitation revenue	Capitation revenue = rate per member per month (PMPM) × members × months, no matter how many services members use.	Multiplying by visits or forgetting the months.	A PMPM rate and a member count.
5	Days in accounts receivable	Days in A/R = accounts receivable ÷ (net patient revenue ÷ 365); fewer days means faster collection.	Using gross charges or dividing by 12, or thinking higher days is better.	An A/R balance and annual revenue.
6	Denial rate	Denial rate = denied claims ÷ claims submitted; tracking the reasons shows which front-end step to fix.	Dividing denied claims by paid claims, or reading a high clean-claim rate as a problem.	Counts of claims submitted and denied.
7	Gross charges vs. net revenue	Net patient revenue = gross charges – contractual adjustments (and other deductions such as charity care); the chargemaster price is not what the hospital collects.	Treating gross charges as revenue.	Charges and contractual allowances in the same stem.
8	Coding compliance	Upcoding (billing a higher-level service than provided) and unbundling (billing parts of a bundled service separately) are fraud risks under the False Claims Act; codes must match documentation.	Believing coding to the highest level is acceptable if the payer does not object.	A code level higher than the documented service, or split billing.

Course scope (public description): Financial Resource Management and Healthcare Reimbursement examines financial practices and reimbursement types within the healthcare industry. This course covers the analysis of regulations required for health reimbursements. This course also covers the evaluation of effective revenue cycle

management, focusing on the organization's financial stability. There are no prerequisites for this course.